

If CPD is suspected, but the baby's head has engaged, a vaginal birth can still be attempted. The labor will be monitored with a [labor graph](#) and if there are signs that the baby is in distress, an emergency Cesarean may be performed. If the head hasn't engaged toward the end of labor, a Cesarean may be offered.

If your doctor suspects CPD in labor, they will reassess the baby's size to check whether they originally underestimated the baby's weight. Even though the combination of a large estimated weight and a slow labor can suggest that there may be delivery problems, often labor proceeds normally.

Inefficient contractions If your labor isn't progressing because your cervix is dilating slowly or has stopped dilating, your doctor will assess the frequency of your contractions, which should be every 2 to 3 minutes. She'll also assess how strong the contractions are by palpating your abdomen: the firmer it feels during contractions, the more likely they are to be effective. If contractions are more widely spaced than they should be and their strength indicates they're unlikely to be effective, they may use one or two techniques to speed up labor, known as augmenting labor. First, they may manually rupture the membranes if they haven't already ruptured, a process called amniotomy, or ARM (see [Breaking the water](#)). This can shorten labor by around one to two hours.

If ARM has no effect, you may be given the drug [oxytocin](#) to increase the strength and frequency of contractions. Initially, a small dose is given and then increased over time until you're having three or four moderately strong contractions every 10 minutes. If this is done, you'll have continuous [electronic fetal monitoring](#) to check that the baby is not distressed by the sudden onset of stronger contractions.

If, several hours after the drugs are started, labor is still not progressing, a Cesarean may be recommended.

Posterior presentation The best position for your baby in labor is an occipito-anterior position with the back of the head (occiput) facing your front. If the back of the head faces your back (occipito-posterior), it can be hard for the baby to turn and move down the birth canal and can prolong labor. The doctor may suggest that you change position to help the baby turn, but the baby usually turns anyway during fully established labor; if not, [forceps](#) or [vacuum](#) may be needed.